

ByteAI

AI-assisted chest X-ray screening for private clinics

Faster triage. Clinic-ready workflow. Clinician always in the loop.

Investor pitch draft | Product V1.0 foundation | Mexico / LATAM focus

Status: working multi-clinic MVP with authenticated roles, durable studies, clinician review, and patient access to finalized reports.

1. The problem

Private clinics capture chest X-rays daily but often wait hours or days for specialist review. Existing PACS and AI tools are expensive, hospital-centric, or stuck at demo stage. Patient communication of results is fragmented (WhatsApp PDFs, paper, no portal). Clinic groups lack a controlled middle layer: one operator, many sites, role-based access.

- Radiologist scarcity outside major hospital systems
- High CXR volume with slow turnaround
- No affordable clinic-group workflow for AI triage + report delivery

2. Why chest X-ray first

CXR is the beachhead. It is high volume, lower cost, and first-line for common thoracic findings. It fits outpatient and private-clinic economics in LATAM better than MRI/CT AI. Adjacent modalities are expansion options after the clinic wedge works - not the starting point.

- Faster path to clinic ROI than advanced imaging AI
- Clear buyer: private clinics and clinic groups
- Decision-support scope reduces early regulatory overclaim risk

3. Solution

ByteAI is clinical screening infrastructure for private clinics: AI-assisted CXR triage plus multi-clinic accounts, auditability, and patient delivery of finalized reports.

- AI probabilities and Grad-CAM heatmaps for trained findings
- Clinician review: impression, recommendations, PDF export
- Roles: Master (platform), Clinic Administrator, Patient
- Durable studies, findings, reports, and access control - not browser-only demos

4. Live product story

- Master creates a clinic, administrator, and patient
- Admin uploads a CXR and runs screening
- Clinician confirms review and finalizes the report
- Patient logs in and sees only finalized reports
- Clinic isolation: Clinic A cannot access Clinic B

5. What is in V1.0 today

Screening	Multi-condition CXR inference + Grad-CAM heatmaps
Workflow UI	Spanish clinical UI, viewer, worklist, PDF report
Security	Auth sessions, CSRF, role-based access control
Tenancy	Multi-clinic memberships and isolation
Persistence	Studies, findings, reports, audit events
Patient access	Read-only portal for finalized reports
Storage	Private local files; Azure Blob interface ready

Not yet in V1.0

- Regulatory clearance / prospective clinical validation
- Hardened production PostgreSQL + Azure deployment
- Email/SMS patient notification
- Billing and subscriptions

Positioning: clinic MVP with real tenancy and workflow - not an FDA-cleared AI radiologist.

6. Future work

Near-term (3-6 months)

- PostgreSQL + Azure Blob production deployment
- Clinic onboarding pack (branding, report templates)
- Notify patient when a report is finalized (link only; no PHI in email)
- Stronger review UX and admin worklist
- Pilots with 2-5 private clinics; measure time-to-review

Mid-term (6-18 months)

- Clinical validation protocol as decision-support
- Additional CXR findings only after performance bar
- Master audit dashboard; retention/compliance policies
- DICOM send / lightweight PACS import

Longer-term (optional)

- Adjacent modalities only after CXR clinic retention and revenue
- White-label / clinic-group marketplace options

7. Business model (draft)

- SaaS per clinic / per site + usage (studies per month)
- Master account for clinic groups and chains
- Premium options: support, branded reports, longer retention

Pricing claims stay conservative until pilot evidence exists.

8. Moat (honest)

Early advantage is workflow and distribution, not unique model weights alone:

- Clinic and patient account graph
- Review / finalize / export audit trail
- Spanish product fit for LATAM private clinics
- Model quality becomes a moat later through local data partnerships

9. How we talk to investors

Say

- Decision support for preliminary triage
- Clinician confirmation required before patient access
- Multi-clinic SaaS foundation already built

Do not say

- Replaces radiologists
- Diagnoses disease autonomously
- Ready for national health systems tomorrow

10. The ask (example framing)

Capital for 3-5 clinic pilots, cloud production, clinical advisory, and 1-2 engineers. Milestones: production deploy, N clinics live, median report turnaround, validation protocol started.

Elevator pitch

ByteAI helps private clinics triage chest X-rays with AI, then finish the job where most tools stop: clinic administration, clinician review, and patient access to finalized reports. Our V1 is a working multi-clinic product with authenticated roles and durable study records. We are raising to put it into production cloud, run paid pilots in Mexico, and start clinical validation - expanding findings only after the clinic workflow proves retention and revenue.

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